Nursing Essentials

Priority: Begin with warmth, breathing, circulation, identification, and safe transition—then reassess the trend.

FOCUS 1

Transition

- **Warmth:** dry and maintain temperature; use skin-to-skin when stable with airway positioning and observation.
- **Breathing:** assess effort, color, and heart rate. Apnea, gasping, or bradycardia requires the neonatal resuscitation pathway.
- **Apgar:** describes condition at set times; do not wait for a score before beginning needed resuscitation.

FOCUS 2

Assess

- **Observe:** activity, tone, feeding, perfusion, respiratory effort, temperature, and the complete newborn examination.
- **Common reference ranges:** respiratory rate 30–60/min and axillary temperature 36.5–37.5°C; interpret with the infant's condition.
- **Prevention:** vitamin K, indicated eye prophylaxis, immunization, and screening follow consent, orders, and current guidance.

FOCUS 3

Feed

- **Support feeding:** cues, latch / milk transfer or safe formula preparation, and an individualized feeding plan.
- **Track:** intake, weight change, voids, stools, and signs of dehydration or jaundice.
- **Risk-based checks:** glucose and other monitoring depend on gestational age, size, maternal history, and symptoms.

Discharge safety

Make it stick: Cold stress increases oxygen and glucose use. Protect temperature while investigating respiratory distress or poor feeding.

Focus	What to know	Nursing application
Safe sleep	supine on a firm, flat, separate sleep surface; keep soft items out of the sleep space.	Teach safe sleep
Teach urgent signs	fever, poor feeding, difficult breathing, unusual lethargy, or worsening jaundice.	Explain urgent symptoms
Complete the plan	screening follow-up, timely newborn visit, safe transport, and caregiver teach-back.	Confirm follow-up

RED FLAGS · ESCALATE PROMPTLY

Apnea, central cyanosis, significant work of breathing, fever, poor perfusion, or unusual lethargy requires prompt newborn assessment.